

BUDGET NARRATIVE: FLORIDA RURAL HEALTH TRANSFORMATION

Introduction and Summary

This Budget Narrative covers the five-year performance period, Federal FY 2026 – FY 2030 (the “project period”) and follows CMS’s *Guidance for Preparing a Budget Request and Narrative*. Throughout this document, we present a **year-by-year breakdown** for each cost category for the entire period of performance. In the remainder of this application, we label years as follows: **Year 1 = FY26, Year 2 = FY27, Year 3 = FY28, Year 4 = FY29, and Year 5 = FY30.**

Per CMS instruction, the budget is developed at the hypothetical **\$200,000,000 per-year** level (total **\$1,000,000,000** over five years); final awards may differ, and the Florida Agency for Health Care Administration (Agency) will rescale initiatives post-award without eliminating or adding initiatives outside the scope of this application. **The budget has been updated to reflect the approved Year 1 allocation of \$209,938,194.50.**

How to Read This Document:

- **Annual Breakdown:** Each cost-category (A-J) provides the annual cost for all five years, in compliance with CMS guidance.
- **Initiative ID:** Each sub-section is explicitly titled with the Initiative number (e.g., “F.1. Initiative 1: Rural and Satellite Clinics”) to link each cost directly to the programmatic framework.
- **Agency vs. Contracted Costs:** This narrative explicitly separates costs and funding administered directly by the Lead Agency (the Agency Program Management Office or “PMO”) from funding that will be **contracted** to implementation partners.
 - **Agency Direct/Indirect Costs** (Sections A, B, C, E, F.16 Admin-A, & J) are for the state-level PMO responsible for administration, oversight, and governance.
 - **Contracted Costs** (Section F.1- F.15 & F.16 Admin-B) encompass *all* programmatic implementation and administrative support, which will be competitively procured and managed by the Agency PMO. This structure directly addresses the requirement to explain how costs are separated and monitored.

Initiative & ID Map: This budget narrative is aligned with the 15 initiatives as numbered and presented in the Project Narrative. These IDs are used for cross-referencing in all budget tables.

- **Initiative 1:** Rural and Satellite Clinics
- **Initiative 2:** Mobile Health
- **Initiative 3:** Community Paramedicine
- **Initiative 4:** Behavioral Health Telehealth & Telehub Psychiatry
- **Initiative 5:** Tele-Specialties & Imaging
- **Initiative 6:** Tele-Intensive Care Unit (eICU)
- **Initiative 7:** Hub-and-Spoke Telestroke
- **Initiative 8:** Workforce Development (CTIO)
- **Initiative 9:** Health and Lifestyle
- **Initiative 10:** Remote Patient Telemonitoring (RPTM)

- **Initiative 11:** Value-Based Purchasing (VBP)
- **Initiative 12:** Retail Clinic Services
- **Initiative 13:** Florida HIE/ENS Onboarding
- **Initiative 14:** Diagnostics Technology Support
- **Initiative 15:** Integrated Medicare-Medicaid Plans Education and Outreach
- **Admin-A:** State PMO Contract Allowance
- **Admin-B:** Regional Collaboratives Administrative Allowance

Award Process: To award funds to applicants, the Agency grouped Florida into four “Super Regions” designed to preserve existing partnerships that rural providers may have with urban centers. Florida’s RHT Program award will be allocated to these four regions based on the percentage of Florida’s rural population in each super region. Each RHT Program initiative will be allocated funding amounts within each super region proportional to the super region’s rural population. The Agency may reserve the option to issue statewide awards when such investments promote efficiency and shared benefit across all regions, when sufficient regional initiatives are not proposed to the Agency.

To distribute RHT Program funds awarded to Florida, the Agency intends to issue a Request for Application (RFA) to notify potential applicants of the available funding. The RFA is a competitive process designed to ensure fair and open competition for public funds. To maximize the impact of RHT Program funds, the Agency will only accept applications from applicants that have formed regional collaboratives within each super region with a single, cohesive, proposal designed to meet the needs of the rural area for which the regional collaborative represents. Each regional collaborative will be comprised predominately of rural providers, with a single entity nominated to represent the consortium. Examples of eligible provider types include hospitals, FQHCs, RHC, practitioners, and pharmacies, among others. Each regional collaborative’s proposal must include, at a minimum, initiatives related to Prevention & Chronic Disease, Consumer Tech Solutions, and Workforce strategies. Due to the size of the four super regions, the Agency may issue awards to multiple applicants within a super region. For all contractors, solicitations will specify:

- **Scope of Work:** Clear deliverables, performance metrics, and milestones tied directly to the Project Narrative
- **Selection Criteria:** Evaluation criteria will include but not be limited to: demonstrated organizational and fiscal capacity to manage a large-scale federal program; experience and existing partnerships with Florida’s rural providers, hospitals, and CBOs; a robust plan for data collection and reporting; strategies for health equity; and cost-reasonableness.
- **Accountability:** Contracts will include requirements for standardized quarterly data submission to the Agency’s Data Dashboard, performance-based payments, and corrective action remedies.

This methodology provides a clear rationale for funding allocations and ensures a competitive, transparent, and accountable selection process for all entities receiving funds. The following table presents potential numbers of awards, per super region, based on maximum and minimum award amounts. This is illustrative only and will change based on Florida’s total RHT Program award:

Potential Awards by Super Region

Super Region	Amount Per Region	Minimum Award Amount	No. Awards at Minimum	Maximum Award Amount	No. Awards at Maximum
NW FL	\$43,864,531	\$5,000,000	8.4	50% of Region Amt	2
NE FL	\$85,585,875	\$10,000,000	8.2	50% of Region Amt	2
SW FL	\$50,182,257	\$5,000,000	9.6	50% of Region Amt	2
SE FL	\$14,980,044	\$2,000,000	7.1	50% of Region Amt	2

A. (Personnel) Salaries and Wages

Personnel Total	\$6,340,000.50	Yearly Overview			Year 1	\$1,268,000.50	
Grant	\$6,340,000				Year 2	\$1,268,000	
Recipient Share	\$0.50				Year 3	\$1,268,000	
					Year4	\$1,268,000	
					Year 5	\$1,268,000	
Position Title	Name	FTE Count	Time	Mths	Annual Salary	Amount Requested	Initiative
Project Director	Antraneise Jackson	1	100%	12	\$154,000	\$154,000	Admin-A
Assistant Director	TBD	1	100%	12	\$120,000	\$120,000	Admin-A
Administrative Professional	TBD	1	100%	12	\$55,000.50	\$55,000.50	Admin-A
Senior Project Manager	TBD	1	100%	12	\$100,000	\$100,000	Admin-A
Project Administrator	TBD	1	100%	12	\$65,000	\$65,000	Admin-A
Budget Analyst	TBD	1	100%	12	\$74,000	\$74,000	Admin-A
Regional Leads	TBD	4	100%	12	\$75,000	\$300,000	Admin-A
Contract/Procurement Lead	TBD	1	100%	12	\$78,000	\$78,000	Admin-A
Data Scientist	TBD	2	100%	12	\$120,000	\$240,000	Admin-A
Grant Compliance Manager	TBD	1	100%	12	\$78,000	\$78,000	Admin-A

This category covers direct salary costs for the 14.0 FTE State Program Management Office (PMO) staff at the Lead Agency (Agency) who will be 100% dedicated to RHTP grant administration and oversight. All programmatic personnel are budgeted under Section F, administered by contractors.

Costs are calculated based on established State of Florida pay grades, are below the federal Executive Level II salary cap, and are 100% allocable to this program's administrative oversight.

Justification:

- Project Director: (Antraneise Jackson):** Serves as the PI/PD, provides executive leadership, chairs the Governance Committee, manages the Administrative/Evaluation Contractor, and serves as the primary CMS liaison. This position directs the overall operation of the project; responsible for overseeing the implementation of project activities, coordination with rural counties, review and approval of materials, provisions of in-service and training, conducting meetings; designs and directs the gathering, tabulating and interpreting of required data; responsible for overall program evaluation

and for staff performance evaluation; and is the responsible authority for ensuring necessary reports/documentation are submitted to CMS. This position relates to all program objectives.

- **Assistant Director:** Supports the Project Director in strategic oversight and operational management of the Rural Health Transformation Program. Assists in coordinating program activities, stakeholder engagement, and cross-functional collaboration. Provides leadership in implementing CMS-defined objectives, monitors progress toward performance metrics and ensures alignment with program goals. Acts as a secondary point of contact for internal and external stakeholders.
- **Administrative Professional:** The Administrative Professional will ensure smooth day-to-day operations of the RHT Program by managing communications, scheduling, documentation, and logistics. This position will serve as the central point of coordination for internal teams and external stakeholders, including CMS, rural health providers, and community organizations.
- **Senior Project Manager:** The Senior Project Manager in the Rural Health Transformation (RHT) Program plays a central role in managing day-to-day operations, ensuring timely execution of activities, and supporting strategic alignment across teams. Project Coordinator also maintains detailed project timelines, task lists, and milestone trackers in addition to coordinating cross-functional activities across clinical, data, finance, and community teams.
- **Project Administrator:** This position supports day-to-day operations in a more general role providing operational and administrative support for project tasks such as documentation, processing, coordination, development of materials, monitor progress toward CMS-defined performance metrics, etc.
- **Budget Analyst:** This position is responsible for financial planning, monitoring, and reporting for the Rural Health Transformation Program. Ensures compliance with CMS financial requirements, tracks expenditures, prepares budget forecasts, and provides analysis to support decision-making. Works closely with leadership to maintain fiscal accountability.
- **Regional Leads:** Serve as primary points of contact for designated regions (NE, NW, SE, SW Florida). Facilitate collaboration among counties, coordinate regional activities, and ensure alignment with program objectives. Engage local stakeholders to implement strategies that improve rural health outcomes and report regional progress to program leadership.
- **Contract/Procurement Lead:** Manages all procurement and contracting activities for the program. Ensures compliance with state and federal regulations, oversees vendor selection and contract execution, and monitors deliverables. Supports program leadership in maintaining transparency and accountability in procurement processes.
- **Data Scientists:** The Data Scientists will lead the design and execution of analytical frameworks that measure progress toward the RHT Program's core objectives: improving healthcare access, supporting rural health innovations, promoting preventive care, and transforming infrastructure. These positions will serve as technical advisors to program leadership and collaborate with clinical, operational, and policy teams. The role is

essential to ensuring Florida's compliance with CMS performance metrics and maximizing the impact of federal investments in rural health.

- **Grant Compliance Manager:** This position ensures adherence to all grant requirements and CMS guidelines. Monitors compliance across program activities, prepares required reports, and provides guidance to staff on regulatory obligations. Works closely with leadership to mitigate risks and maintain program integrity.

B. Fringe Benefits

Benefits Total	\$3,071,540	Yearly Overview	Year 1	\$614,308
Grant	\$3,071,540		Year 2	\$614,308
Recipient Share	\$0		Year 3	\$614,308
			Year 4	\$614,308
			Year 5	\$614,308

Position Title	Name	Retirement	Social Security	Health Insurance	Life Insurance	Disability	FTE	Cost	Initiative
Program Director	Antraneise Jackson	\$21,606	\$11,475	\$24,186	\$43	\$60	1	\$57,370	Admin-A
Assistant Director	TBD	\$16,836	\$9,180	\$24,186	\$43	\$48	1	\$50,293	Admin- A
Admin Professional	TBD	\$7,717	\$4,208	\$24,186	\$43	\$22	1	\$36,176	Admin- A
Senior Project Manager	TBD	\$14,030	\$7,650	\$24,186	\$43	\$40	1	\$45,949	Admin-A
Program Administrator	TBD	9,120	\$4,973	\$24,186	\$43	\$26	1	\$38,348	Admin- A
Budget Analyst	TBD	\$10,943	\$5,967	\$24,186	\$43	\$31	1	\$41,170	Admin-A
Regional Leads	TBD	\$42,090	\$22,950	\$96,744	\$172	\$120	4	\$162,076	Admin-A
Contract/Procurement Lead	TBD	\$10,943	\$5,967	\$24,186	\$43	\$31	1	\$41,170	Admin-A
Data Scientists	TBD	\$33,672	\$18,360	\$48,372	\$86	\$96	2	\$100,586	Admin-A
Grants Compliance Manager	TBD	\$10,943	\$5,967	\$24,186	\$43	\$31	1	\$41,170	Admin-A

Justification: The Fringe Benefit Costs are the standard rates and payments applied to each full-time equivalent position within the State of Florida employment system. The retirement rate in the table is based on the employer contribution rate as published by the Florida Division of Retirement and is set at 14.03% of gross salary. The FICA rate is composed of the Social Security rate of 6.20% and the Medicare rate of 1.45% for a combined rate of 7.65%. Florida provides a state/employer share of the State Group Health Insurance premiums on behalf of employees with benefits each fiscal year. The state share of the health care premium is set annually in the General Appropriations Act and is currently \$2015.48 per month per person. The state share of the premium is estimated at the same rate for all five years for this application. Life Insurance premiums are \$3.58, and disability is provided at a rate of .04% of an FTE gross salary.

C. Travel

Travel Total	\$52,444	Yearly Overview		Year 1	\$10,489
Grant	\$52,444			Year 2	\$10,489
Recipient Share	\$0			Year 3	\$10,489
				Year 4	\$10,489
				Year 5	\$10,489
Purpose of Travel	Location	Item	Rate	Cost	Initiative
Site Visits	Various Rural Sites	Car Rental	$27.32 \times 2 \text{ staff} \times (2 \text{ trips} \times 3 \text{ days} + 5 \text{ trips} \times 2 \text{ days} + 5 \text{ trips} \times 1 \text{ day})$	\$574	Admin
		Lodging	$160 \times 2 \text{ staff} \times (2 \text{ trips} \times 2 \text{ nights} + 5 \text{ trips} \times 1 \text{ night} + 5 \text{ trips} \times 0 \text{ nights})$	\$2,880	Admin
		Per Diem (includes meals)	Per diem: \$80 non-lodging day, \$36 lodging day; per staff per trip.	\$2,568	Admin
Statewide Stakeholder Meetings	4 Rotating Locations	Car Rental	$27.32 \times 3 \text{ staff} \times 4 \text{ trips/year} \times 2\text{-day duration}$	\$219	Admin
		Lodging	$160 \times 3 \text{ staff} \times 4 \text{ trips/year} \times 1\text{-night stay}$	\$1,920	Admin
		Per Diem (includes meals)	Same formula as above	\$1,392	Admin
Contingency Site Meetings	Statewide	Car Rental	$27.32 \times 2 \text{ staff} \times 5 \text{ trips/year} \times 1\text{-day duration}$	\$137	Admin
		Per Diem (includes meals)	Same formula as above	\$800	Admin
Baltimore Summit	Annual	Car Rental	$80 \times 4 \text{ staff (round trip)}$	\$160	Admin
		Lodging	$150 \times 4 \text{ staff (3 nights)}$	\$1,800	Admin
		Airline	$482 \times 4 \text{ staff (round trip)}$	\$1,928	Admin
		Uber/Lyft/taxi	$56.5 \times 4 \text{ staff}$	\$226	Admin
		Per Diem (with meals)	Same formula as above	\$752	Admin

Justification: This category funds all travel for the 4.0 FTE state-level PMO staff (Section A) to conduct mandatory grant oversight, contractor monitoring, and stakeholder engagement. Costs are calculated based on GSA rates for mileage, lodging, and per diem and are allocated as follows:

- **Site Visits & Contractor Monitoring:** This is essential for the Agency's oversight responsibilities (2 CFR 200.332). It funds travel for the 4.0 FTEs to conduct at least quarterly site visits to the four Regional Lead Contractors and periodic site visits to their downstream implementation sites across 31 rural counties.
- **Stakeholder Meetings:** Covers PMO travel to the statewide stakeholder meetings (Quarterly in Y1, Bi-Annually in Y2-Y5).
- **Contingency Site Meetings:** Allocates funds for required travel to CMS-mandated meetings.
- **Mandatory CMS RHTP Summit:** Covers travel for the staff to attend the mandatory CMS Rural Health Transformation Summit in Baltimore on March 18, 2026.

D. Equipment

Annual Equipment Budget: \$0 (Years 1-5)

Initiative: N/A

Justification: Florida requests \$0 under the SF-424A Equipment category for the Lead Agency. This is a deliberate structural choice to ensure clean federal reporting and compliance.

All programmatic equipment (defined as unit cost $\geq$ \$10,000 or entity's capitalization threshold), such as telehealth carts, RPTM kits, and eICU hardware, will be **provided via competitively procured contracts** and reflected in **Section F (Contractual Costs)**.

E. Supplies

		Yearly Overview	
Supplies Total	\$408,800	Year 1	\$81,760
Grant	\$408,800	Year 2	\$81,760
Recipient Share	\$0	Year 3	\$81,760
		Year 4	\$81,760
		Year 5	\$81,760

Supplies	Rate	Total Annual Cost	Initiatives
Postage	12 months x \$11.75/mo x 14 staff	\$1,974	Admin-A
Printing & Reproduction	12 months x \$10.08/mo x 14 staff	\$1,694	Admin-A
Office Supplies	12 months x \$32.08/mo x 14 staff	\$5,390	Admin-A
Software and Training	12 months x \$225.92/mo x 14 staff	\$37,954	Admin-A
Computing Equipment	12 months x \$206.83/mo x 14 staff	\$34,748	Admin-A

Justification: This category includes administrative supplies for the State PMO only.

- **State PMO Admin Supplies:** Funds standard office consumables, software & training, and computing equipment for State PMO staff only.
- All programmatic supplies (e.g., RPTM consumables, clinical supplies, mobile unit fuel) are budgeted within the contractor costs in Section F, as they are procured directly by the implementation partners as part of their contractual scope of work.

F. Contractual Costs by Initiative

Total Annual Contractual Budget: \$207,963,637 (Years 1-5)

This category includes all programmatic and administrative support costs executed via contracts. This section's total represents the total from the 15 Programmatic Initiatives (F.1- F.15) executed across the regional collaboratives, admin allowance awarded to contractors across the regional collaboratives (F.16 Admin-B), and contracts executed by the State PMO (F.16 Admin-A).

The initiative tables in this section represent the full, all-inclusive annual cost of each initiative, including all contractor-procured personnel, travel, equipment, supplies, and other programmatic expenses. The cost in F.1 – F.15 for the initiatives, are reflective of research, stakeholder outreach, and responses to the Agency's request for information. The Agency has allocated the costs across the following categories: Technology & Support, Supplies, and Personnel. The Agency will update the budget detail post award prior to incurring cost or drawing down funds.

F1. Initiative 1: Rural and Satellite Clinics

Annual Total:	\$25,000,000	5Y Total:	\$125,000,000	
		Grant:	\$125,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 17,500,000	FY26 - FY30	\$ 87,500,000	Initiative 1
Supplies	\$ 2,500,000	FY26 - FY30	\$ 12,500,000	Initiative 1
Personnel	\$ 5,000,000	FY26 - FY30	\$ 25,000,000	Initiative 1

Contractors: Contractors (TBD via competitive RFA).

Justification: This initiative provides start-up funding for established urban physician or dental practices to expand their services into rural areas by opening new or satellite offices. Funding also supports nurse practitioner and physician assistant–led clinics operating within scope and under appropriate supervision, enhancing access to care for rural residents. These clinics offer primary care, dental, chronic disease management, obstetric and gynecologic care, and mental health services, promoting preventive care and early intervention. Health Information Exchange HIE and Encounter Notification Service (ENS) integration will improve continuity of care, follow-up, and outcome tracking.

Funding Support:

- Start-up and expansion costs, including leasing, equipment, and initial supplies
- Health Information Technology such as Electronic Health Record (EHR) connections, tele-supervision tools, and cybersecurity upgrades to support care delivery
- Workforce training, staff recruitment onboarding initial costs, and supervision costs (time-limited)
- Care coordination and navigation functions to connect patients with services, consistent with the Notice of Funding Opportunity limits on provider payments

F2. Initiative 2: Mobile Health

Annual Total:	\$20,000,000	5Y Total:	\$100,000,000	
		Grant:	\$100,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 16,000,000	FY26 - FY30	\$ 80,000,000	Initiative 2
Supplies	\$ 1,000,000	FY26 - FY30	\$ 5,000,000	Initiative 2
Personnel	\$ 3,000,000	FY26 - FY30	\$ 15,000,000	Initiative 2

Contractors: Contractors (TBD via competitive RFA).

Justification: The Mobile Health initiative brings preventive, screening, rehabilitation, and prenatal services directly to rural communities on scheduled routes. Mobile units reduce missed care caused by transportation barriers and integrate with local hospitals, FQHCs, RHCs, County

Health Departments, and community-based organizations to ensure continuity of care. Services include chronic disease screenings, prenatal visits, and patient education. Real-time data exchange through secure EHR connections ensures timely documentation and follow-up care.

Funding Support:

- Operational costs for leased mobile units, staff, and supplies used to deliver on-site services; the grant will not pay for vehicle purchases or new construction
- Health Information Technology tools including tele-supervision, mobile connectivity, and secure electronic health record access for same-day documentation and data exchange
- Workforce and training expenses for clinicians and community health workers supporting mobile care and follow-up
- Quality assurance, route scheduling, and communication tools such as patient reminders and closed-loop referral systems

F3. Initiative 3: Community Paramedicine

Annual Total:		\$18,000,000	5Y Total:		\$90,000,000
			Grant:		\$90,000,000
			Recipient Share:		\$0.00
Category	Annual Cost	Years	Total	Initiative	
Technology & Equipment	\$ 8,100,000	FY26 - FY30	\$ 40,500,000	Initiative 3	
Supplies	\$ 1,800,000	FY26 - FY30	\$ 9,000,000	Initiative 3	
Personnel	\$ 8,100,000	FY26 - FY30	\$ 40,500,000	Initiative 3	

Contractors: Contractors (TBD via competitive RFA).

Justification: This initiative deploys community paramedics and, where appropriate, other licensed practitioners to provide on-site support for minor illnesses within their scope of practice and deliver post-discharge care for high-risk patients to reduce preventable readmissions. When appropriate, telehealth services will be used to support patient care, and hospitals and EMS will collaborate through the Event Notification System to ensure timely follow-up and coordination with primary care teams. Activities include medication reconciliation, health education, and coordination with primary care providers and behavioral health specialists.

Funding Support:

- Operations for post-discharge and home-visit teams including training, equipment, safety supplies, and program management
- Health Information Technology connections between electronic Patient Care Records (ePCR) and electronic health records, Event Notification Service alerts, and two-way texting platforms
- Remote Patient Telemonitoring kits and Artificial Intelligence–assisted decision support for patient follow-up
- Training and workforce development for paramedics, registered nurses, and community health workers providing in-home stabilization and care coordination

F4. Initiative 4: Behavioral Health Telehealth & Telehub Psychiatry

Annual Total:	\$18,000,000	5Y Total:	\$90,000,000	
		Grant:	\$90,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 9,900,000	FY26 - FY30	\$ 49,500,000	Initiative 4
Supplies	\$ 1,800,000	FY26 - FY30	\$ 9,000,000	Initiative 4
Personnel	\$ 6,300,000	FY26 - FY30	\$ 31,500,000	Initiative 4

Contractors: Contractors (TBD via competitive RFA).

Justification: The Behavioral Health Telehealth and Telehub Psychiatry initiative establishes regional centers that connect rural residents to same-day psychiatric consultations, therapy, and coordinated care. Through the Collaborative Care Model, psychiatrists and behavioral health specialists support local primary care providers with shared treatment plans and medication management. Activities include implementing telepsychiatry technology, workforce training on evidence-based protocols, and using standardized screening tools such as Patient Health Questionnaire-9 (PHQ-9), Generalized Anxiety Disorder-7 (GAD-7), and Edinburgh Postnatal Depression Scale (EPDS) to track outcomes and ensure timely, data-driven interventions.

Funding Support:

- Staffing and operations for Telehub care managers, referral coordinators, and consulting psychiatrists who support primary care teams through shared care plans
- Telepsychiatry technology, registry tools to track outcomes such as Patient Health Questionnaire-9 (PHQ-9), Generalized Anxiety Disorder-7 (GAD-7), and Edinburgh Postnatal Depression Scale (EPDS) scores, and secure EHR integration
- Workforce training for clinicians and care managers on measurement-based care, privacy, and telehealth protocols
- Software licenses, equipment, and minor facility upgrades consistent with Notice of Funding Opportunity (NOFO) capital-use restriction

F5. Initiative 5: Tele-Specialties & Imaging

Annual Total:	\$16,000,000	5Y Total:	\$80,000,000		
		Grant:	\$80,000,000		
		Recipient Share:	\$0.00		
Category	Annual Cost	Years	Total	Initiative	
Technology & Equipment	\$ 12,000,000	FY26 - FY30	\$ 60,000,000	Initiative 5	
Supplies	\$ 800,000	FY26 - FY30	\$ 4,000,000	Initiative 5	
Personnel	\$ 3,200,000	FY26 - FY30	\$ 16,000,000	Initiative 5	

Contractors: Contractors (TBD via competitive RFA).

Justification: This initiative enables rural hospitals and clinics to host virtual specialty clinics, allowing patients to consult with specialists like cardiologists, pulmonologists, endocrinologists, oncologists, and neurologists via telehealth. Local nurses support physical exams and transmit

test results securely for prompt review. Cloud-based imaging systems and Picture Archiving and Communication System (PACS) integration facilitate rapid diagnostics and specialist feedback. The initiative aims to reduce unnecessary travel, accelerate diagnosis, and improve chronic disease outcomes in rural communities.

Funding Support:

- Operational costs for virtual specialty clinic days and eConsult sessions at rural hospitals or clinics, including nursing support and scheduling systems
- Cloud-based imaging systems and Picture Archiving and Communication System (PACS) integration for secure data exchange
- Workforce training on tele-specialty workflows and diagnostic coordination
- Cybersecurity enhancements and Health Information Technology connections to support data interoperability

F.6. Initiative 6: Tele-Intensive Care Unit (eICU)

Annual Total:	\$14,000,000	5Y Total:	\$70,000,000	
		Grant:	\$70,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 10,500,000	FY26 - FY30	\$ 52,500,000	Initiative 6
Supplies	\$ 700,000	FY26 - FY30	\$ 3,500,000	Initiative 6
Personnel	\$ 2,800,000	FY26 - FY30	\$ 14,000,000	Initiative 6

Contractors: Contractors (TBD via competitive RFA).

Justification: The Tele-Intensive Care Unit (eICU) initiative enables rural hospitals to connect with remote critical care physicians and nurses who provide 24/7 patient monitoring through secure video and data systems. This initiative enhances care for critically ill patients by allowing early intervention, reducing unnecessary transfers, and keeping families close to home. It supports virtual nursing, medication management, and patient-family communication. Additionally, it integrates with electronic health record (EHR) systems to ensure timely response and continuity of care.

Funding Support:

- Tele-ICU and tele-hospitalist platforms, monitoring software, and secure communication system
- Integration of alert and order-set tools into existing hospital electronic health records to enhance clinical response and patient safety
- Workforce training for hospital staff on remote monitoring, escalation protocols, and telehealth quality standards
- Minor equipment upgrades and facility modifications needed for telehealth operations, within capital-use limits

F7. Initiative 7: Hub-and-Spoke Telestroke

Annual Total:	\$10,000,000	5Y Total:	\$50,000,000	
		Grant:	\$50,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 8,000,000	FY26 - FY30	\$ 40,000,000	Initiative 7
Supplies	\$ 1,000,000	FY26 - FY30	\$ 5,000,000	Initiative 7
Personnel	\$ 1,000,000	FY26 - FY30	\$ 5,000,000	Initiative 7

Contractors: Contractors (TBD via competitive RFA).

Justification: The Hub-and-Spoke Telestroke initiative connects rural emergency departments with stroke specialists via secure, real-time video for immediate assessment and treatment. Computed tomography (CT) scans are transmitted electronically to remote neurologists for rapid review and diagnosis. Standardized treatment protocols guide timely administration of clot-dissolving medication and decisions about transfer or observation. This initiative saves lives by ensuring rural hospitals can deliver evidence-based stroke care locally.

Funding Support:

- Technology infrastructure for on-call tele-neurology consults, including video connectivity, CT image transfer, and real-time communication
- Integration of electronic health record, Picture Archiving and Communication System, and Electronic Notification Service systems to ensure rapid diagnosis, follow-up, and data exchange
- Training and simulation exercises for hospital and emergency medical staff on stroke and emergency pathways
- Quality monitoring tools for treatment times, transfers, and outcome tracking

F8. Initiative 8: Workforce Development (CTIO)

Annual Total:	\$12,683,313	5Y Total:	\$63,416,565	
		Grant:	\$63,416,565	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 634,166	FY26 - FY30	\$ 3,170,830	Initiative 8
Supplies	\$ 634,165	FY26 - FY30	\$ 3,170,825	Initiative 8
Personnel	\$ 11,414,982	FY26 - FY30	\$ 57,074,910	Initiative 8

Contractors: Contractors (TBD via competitive RFA).

Justification: The Clinical Training Investment Opportunity (CTIO) initiative strengthens Florida's rural healthcare pipeline by supporting clinical training programs, supervised rotations, and practitioner recruitment and retention efforts. It funds housing assistance for trainees, professional development, and career pathways through local high schools and community

colleges. Participants commit to a five-year service period in rural communities, ensuring long-term workforce stability and access to essential healthcare services.

Funding Support:

- Clinical training sites, preceptorships, and supervised rotations for nursing, medical, and behavioral health trainees in rural areas
- Recruitment and retention incentives that comply with the required five-year rural service commitment
- Workforce housing supports and professional development for community health workers and emergency medical staff
- Technology and tracking systems for workforce placement, retention, and evaluation
- Training/curriculum development costs

F9. Initiative 9: Health and Lifestyle

Annual Total:	\$10,130,531	5Y Total:	\$50,652,655	
		Grant:	\$50,652,655	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 3,545,686	FY26 - FY30	\$ 17,728,430	Initiative 9
Supplies	\$ 2,532,633	FY26 - FY30	\$ 12,663,165	Initiative 9
Personnel	\$ 4,052,212	FY26 - FY30	\$ 20,261,060	Initiative 9

Contractors: Contractors (TBD via competitive RFA).

Justification: The Health and Lifestyle initiative addresses food insecurity and nutrition-related health disparities by integrating food screenings, brief nutrition counseling, and referrals to local food programs and pantries into clinical settings. It also supports physician and advanced practice provider education in nutrition counseling, works with school districts to promote physical activity through the Presidential Fitness Test, and supports nutrition classes in middle and high schools. Together, these strategies improve nutrition, encourage healthy behaviors, and reduce chronic disease in rural populations.

Funding Support:

- Screening tools and referral systems embedded in clinics to identify food insecurity and nutrition-related risks
- Closed-loop referral systems that track closure of social related health needs, like food insecurity, including workflow integration with existing electronic health record systems
- Digital tracking tools for referrals to community food programs such as Fresh Access Bucks, Feeding Florida, and Farm Share, and follow-up outcomes like blood pressure and Hemoglobin A1c levels
- Training/curriculum development costs
- Equipment and supplies

F.10. Initiative 10: Remote Patient Telemonitoring (RPTM)

Annual Total:	\$14,000,000	5Y Total:	\$70,000,000	
		Grant:	\$70,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 11,900,000	FY26 - FY30	\$ 59,500,000	Initiative 10
Supplies	\$ 700,000	FY26 - FY30	\$ 3,500,000	Initiative 10
Personnel	\$ 1,400,000	FY26 - FY30	\$ 7,000,000	Initiative 10

Contractors: Contractors (TBD via competitive RFA).

Justification: The Remote Patient Telemonitoring initiative allows rural patients with chronic conditions such as hypertension, diabetes, heart failure, and chronic obstructive pulmonary disease (COPD) to use connected devices like blood pressure cuffs, glucose meters, and scales to transmit health data directly to care teams. An artificial intelligence–driven triage system identifies patients at risk and triggers timely nurse follow-ups, preventing emergency department visits and hospitalizations.

Funding Support:

- Cellular-enabled monitoring kits (blood pressure cuffs, scales, glucometers, pulse oximeters) and related data platforms
- Artificial Intelligence–driven triage tools and dashboards for continuous patient monitoring and escalation
- Staffing and clinical oversight for remote monitoring, data interpretation, and patient follow-up
- Cybersecurity and Health IT upgrades to support safe data transmission and storage

F.11. Initiative 11: Value-Based Purchasing (VBP)

Annual Total:	\$10,000,000	5Y Total:	\$50,000,000	
		Grant:	\$50,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 4,000,000	FY26 - FY30	\$ 20,000,000	Initiative 11
Supplies	\$ 2,000,000	FY26 - FY30	\$ 10,000,000	Initiative 11
Personnel	\$ 4,000,000	FY26 - FY30	\$ 20,000,000	Initiative 11

Contractors: Contractors (TBD via competitive RFA).

Justification: The Value-Based Purchasing initiative transitions rural healthcare providers from a fee-for-service model to performance-based payment models that reward quality, outcomes, and efficiency. It begins with incentive payments for achieving quality benchmarks, progresses to shared savings arrangements, and eventually includes shared risk models. The initiative also supports rural practices in obtaining Patient-Centered Medical Home (PCMH) designation, improving care coordination and chronic disease management. This approach builds provider capacity to deliver high-value care and ensures financial sustainability for rural facilities.

Funding Support:

- Training and coaching for rural providers to learn how value-based care works and how to track performance measures
- Software, data tools, and technology to help clinics report results and compare progress
- Consultants or staff support to help design fair payment models and set achievable goals
- Shared technology systems that let clinics coordinate care, manage patient populations, and exchange data securely
- Training for healthcare workers on care coordination, recordkeeping, and connecting payments to improved patient outcomes

F.12. Initiative 12: Retail Clinic Services

Annual Total:		\$6,000,000	5Y Total:		\$30,000,000
			Grant:		\$30,000,000
			Recipient Share:		\$0.00
Category	Annual Cost	Years	Total	Initiative	
Technology & Equipment	\$ 4,500,000	FY26 - FY30	\$ 22,500,000	Initiative 12	
Supplies	\$ 300,000	FY26 - FY30	\$ 1,500,000	Initiative 12	
Personnel	\$ 1,200,000	FY26 - FY30	\$ 6,000,000	Initiative 12	

Contractors: Contractors (TBD via competitive RFA).

Justification: The Retail Clinic Services initiative expands access to convenient healthcare by reimbursing rural pharmacies that provide non-emergency medical services within their scope of practice. Licensed providers offer acute and preventive care visits on-site, including diagnostic testing, and chronic disease monitoring. Diagnostic telehealth kiosks also connect patients with remote clinicians for same-day assessment and treatment. This initiative leverages existing pharmacy infrastructure to address care gaps and reduce unnecessary emergency department visits.

Funding Support:

- Setup of private, protocol-based clinic areas within pharmacies for basic, non-emergency health assessments
- Health IT connections between pharmacy systems and local EHRs to support referrals and care continuity
- Workforce training for pharmacists and advanced practice providers on clinical documentation and telehealth integration
- Cybersecurity and patient privacy safeguards consistent with the Notice of Funding Opportunity standards

F.13. Initiative 13: Florida HIE/ENS Onboarding

Annual Total:	\$5,000,000	5Y Total:	\$25,000,000	
		Grant:	\$25,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 4,750,000	FY26 - FY30	\$ 23,750,000	Initiative 13
Supplies	\$ 125,000	FY26 - FY30	\$ 625,000	Initiative 13
Personnel	\$ 125,000	FY26 - FY30	\$ 625,000	Initiative 13

Contractors: Contractors (TBD via competitive RFA).

Justification: This initiative funds onboarding for rural hospitals, clinics, and providers to the Florida Health Information Exchange (HIE) and Encounter Notification Service (ENS). By connecting providers to real-time admit, discharge, and transfer alerts, the program enhances care coordination, reduces readmissions, and promotes seamless transitions between care settings. Funding supports the modernization of health information platforms, including interface setup, data standardization, and staff training to ensure full participation and compliance with State interoperability standards.

Funding Support:

- Provider onboarding to Florida's Health Information Exchange and ENS network, including setup fees and staff training
- Interface development to connect electronic health records using Health Level 7 (HL7) and Fast Healthcare Interoperability Resources (FHIR) standards
- Workflow implementation for discharge alerts, care coordination, and follow-up tracking
- Cybersecurity, data integrity, and interoperability testing to ensure compliance with state standards

F.14. Initiative 14: Diagnostics Technology Support

Annual Total:	\$9,130,531	5Y Total:	\$45,652,655	
		Grant:	\$45,652,655	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 8,217,478	FY26 - FY30	\$ 41,087,390	Initiative 14
Supplies	\$ 456,527	FY26 - FY30	\$ 2,282,635	Initiative 14
Personnel	\$ 456,527	FY26 - FY30	\$ 2,282,635	Initiative 14

Contractors: Contractors (TBD via competitive RFA).

Justification: The Diagnostics Technology Support initiative equips rural hospitals, clinics, and laboratories with cloud-based Picture Archiving and Communication Systems (PACS) to enable rapid sharing and review of diagnostic images and laboratory results. The initiative enhances connectivity between rural and specialty facilities, improving turnaround times and diagnostic accuracy. It supports workforce training, cybersecurity, and EHR integration to ensure secure, efficient data exchange and faster clinical decision-making.

Funding Support:

- Provider onboarding to Florida's Health Information Exchange and ENS network, including setup fees and staff training
- Interface development to connect electronic health records using Health Level 7 (HL7) and Fast Healthcare Interoperability Resources (FHIR) standards
- Workflow implementation for discharge alerts, care coordination, and follow-up tracking
- Cybersecurity, data integrity, and interoperability testing to ensure compliance with state standards

F.15. Initiative 15: Integrated Medicare-Medicaid Plans Education and Outreach

Annual Total:	\$1,000,000	5Y Total:	\$5,000,000	
		Grant:	\$5,000,000	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Technology & Equipment	\$ 50,000	FY26 - FY30	\$ 250,000	Initiative 15
Supplies	\$ 50,000	FY26 - FY30	\$ 250,000	Initiative 15
Personnel	\$ 900,000	FY26 - FY30	\$ 4,500,000	Initiative 15

Contractors: Contractors (TBD via competitive RFA).

Justification: The Integrated Medicare-Medicaid Plans Education and Outreach initiative increases awareness and participation among Floridians eligible for both Medicare and Medicaid. It funds outreach, education, and stakeholder training to promote enrollment in integrated plan models that combine benefits and improve care coordination. Florida currently has 916,577 dual-eligible residents, including 424,049 full-dual eligibles who receive full benefits from both programs. Of all dual-eligible individuals, 70.7 % (648,224 people) are enrolled in an integrated Medicaid-Medicare plan, and 64.2 % (272,273 full-dual eligibles) are in an integrated plan. Florida's RHTP will provide additional support to increase enrollment in integrated plans in rural areas, helping more beneficiaries join integrated plans that simplify access to care, reduce duplication, and improve outcomes for people with complex medical and social needs.

Funding Support:

- Supports outreach, education, and stakeholder training to increase integrated plan enrollment

F.16. Initiative Admin-A: State PMO Contracts

Annual Total:	\$13,350,931	5Y Total:	\$66,754,655	
		Grant:	\$66,754,655	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
Contractual	\$13,350,931	FY26 - FY30	\$66,754,655	Admin-A

Contractors: Contractors (TBD via competitive RFA).

Justification: The State PMO contract funds will be used to support the administration of the RHTP. This amount is the remaining administrative allowance after accounting for the States Direct Cost in A, B, C, & E, combined with the 3% Admin allowance to the 15 initiatives. The state plans to procure a Grant Management System (GMS) with a portion of these funds. The remaining funds will be used as vendor support to administer the program.

F.16. Initiative Admin-B: Administrative Costs by Super Region

Annual Total:	\$5,668,331	5Y Total:	\$28,341,655	
		Grant:	\$28,341,655	
		Recipient Share:	\$0.00	

Super Region	Amount Per	Years	Total	Initiative
NW FL	\$1,277,608	FY26 - FY30	\$6,388,040	Admin-B
NE FL	\$2,492,792	FY26 - FY30	\$12,463,960	Admin-B
SW FL	\$1,461,619	FY26 - FY30	\$7,308,095	Admin-B
SE FL	\$436,312	FY26 - FY30	\$2,181,560	Admin-B

Contractor: Contractors (TBD via competitive RFA).

Scope: Provide program administration support for each of the Super Regions.

Method of Selection & Accountability: Admin-B amounts equal 3% of each Super Region's Initiative total. Each applicant will receive 3% for administrative cost upon award.

Justification: This contract is for administrative support and is counted toward the 10% administrative cap, per RHTP FAQ Q67.

G. Construction

Annual Construction Budget: \$0 (Years 1-5)

Initiative: N/A

Justification: Not Applicable.

H. Other

Annual Other Budget: \$0 (Years 1-5)

Initiative: N/A

Justification: No direct applicant costs are requested by the State Agency in this category. All programmatic costs that might typically fall under "Other" (e.g., software licenses, data subscriptions, provider stipends, communication expenses, participant incentives, training materials, subscription fees for data services like HIE/ENS, non-capitalized minor alterations) are essential components of the 15 initiatives. These costs are appropriately budgeted, procured, and managed by the contractors detailed in Section F and are justified within each initiative's detailed budget breakdown.

I. Total Direct Costs

Annual Total:	\$209,436,264.50	5Y Total:	\$1,047,181,320.50	
		Grant:	\$1,047,181,320.50	
		Recipient Share:	\$0.00	
Category	Annual Cost	Years	Total	Initiative
A. Personnel (State)	\$1,268,000.50	FY26 - FY30	\$6,340,000	Admin
B. Fringe (State)	\$614,308	FY26 - FY30	\$3,071,540	Admin
C. Travel (State)	\$10,489	FY26 - FY30	\$52,445	Admin
D. Equipment (State)	\$0	FY26 - FY30	\$0	
E. Supplies (State)	\$81,760	FY26 - FY30	\$408,800	Admin
F. Contractual	\$207,461,707	FY26 - FY30	\$1,037,308,535	1-15 & Admin
G. Construction	\$0	FY26 - FY30	\$0	
H. Other (State)	\$0	FY26 - FY30	\$0	

Justification: This represents the sum of all direct costs across categories A through H for the entire project annually.

J. Indirect Costs (State Agency Only)

Annual Total:		\$501,930	5Y Total:		\$2,509,651
			Grant:		\$2,509,651
			Recipient Share:		\$0.00
Base	Rate	Initiative	Annual Cost	Years	Total
(See Justification)	(Per NICRA)	Admin-A	\$501,930	FY26 - FY30	\$2,509,651

Justification: This requested amount of \$501,930 annually is based on the State of Florida Agency for Health Care Administration's current, federally approved Negotiated Indirect Cost Rate Agreement (NICRA), a copy of which is attached. This amount is calculated by applying the approved rate to the appropriate Modified Total Direct Cost (MTDC) base, which consists of the Agency's direct administrative costs (Sections A & B). Indirect costs for all contractors are included in their contract totals in Section F.

Administrative Cost Cap Attestation: This amount is also precisely calculated to ensure total administrative costs do not exceed the 10% statutory cap.

- **Total Direct Administrative Costs:**
 - State PMO Direct (A+B+C+E): \$1,974,557.50
 - Admin Contractor (F.16 Admin-A+ F.16 Admin-B): \$18,517,332
 - Subtotal Direct Admin: \$20,491,889
- **Total Administrative Costs:**
 - Direct Admin: \$20,491,889.50
 - State Indirect (J): \$501,930
 - Total Annual Admin: \$20,993,819.50
- **Compliance:** This total of \$20,993,819.50 represents exactly 10.0% of the

\$209,938,194.50 Year 1 budget and is in full compliance with the statutory cap.

Total Year 1 Budget Request: \$209,436,264.50 (Direct) + \$501,930 (Indirect) = \$209,938,194.50

BUDGET INFORMATION - Non-Construction Programs

OMB Approval No. 0348-0044

SECTION A - BUDGET SUMMARY

Grant Program Function or Activity (a)	Catalog of Federal Domestic Assistance Number (b)	Estimated Unobligated Funds		New or Revised Budget		
		Federal (c)	Non-Federal (d)	Federal (e)	Non-Federal (f)	Total (g)
1. Rural Health Transfor	CMS-RHT-26-001	\$	\$	\$ 209,938,194.50	\$	\$ 209,938,194.50
2.						0.00
3.						0.00
4.						0.00
5. Totals		\$ 0.00	\$ 0.00	\$ 209,938,194.50	\$ 0.00	\$ 209,938,194.50

SECTION B - BUDGET CATEGORIES

6. Object Class Categories	GRANT PROGRAM, FUNCTION OR ACTIVITY				Total (5)
	(1)	(2)	(3)		
a. Personnel	\$ 1,263,580.50	\$	\$	\$	\$ 1,263,580.50
b. Fringe Benefits	614,308.00				614,308.00
c. Travel	14,909				14,909
d. Equipment	0.00				0.00
e. Supplies	81,760.00				81,760.00
f. Contractual	207,461,707.00				207,461,707.00
g. Construction	0.00				0.00
h. Other	0.00				0.00
i. Total Direct Charges (sum of 6a-6h)	209,436,264.50	0.00	0.00	0.00	209,436,264.50
j. Indirect Charges	501,930.00				501,930.00
k. TOTALS (sum of 6i and 6j)	\$ 209,938,194.50	\$ 0.00	\$ 0.00	\$ 0.00	\$ 209,938,194.50
7. Program Income	\$	\$	\$	\$	\$ 0.00

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SECTION C - NON-FEDERAL RESOURCES					
(a) Grant Program	(b) Applicant	(c) State	(d) Other Sources	(e) TOTALS	
8. Rural Health Transformation Program	\$ 209,938,194.50	\$	\$	\$ 209,938,194.50	
9.				0.00	
10.				0.00	
11.				0.00	
12. TOTAL (sum of lines 8-11)	\$ 209,938,194.50	\$ 0.00	\$ 0.00	\$ 209,938,194.50	
SECTION D - FORECASTED CASH NEEDS					
	Total for 1st Year	1st Quarter	2nd Quarter	3rd Quarter	4th Quarter
13. Federal	\$ 0.00	\$	\$	\$	\$
14. Non-Federal	0.00				
15. TOTAL (sum of lines 13 and 14)	\$ 0.00	\$ 0.00	\$ 0.00	\$ 0.00	\$ 0.00
SECTION E - BUDGET ESTIMATES OF FEDERAL FUNDS NEEDED FOR BALANCE OF THE PROJECT					
(a) Grant Program	FUTURE FUNDING PERIODS (Years)				
	(b) First	(c) Second	(d) Third	(e) Fourth	
16. Rural Health Transformation Program	\$ 209,938,194.50	\$	\$	\$	
17.					
18.					
19.					
20. TOTAL (sum of lines 16-19)	\$ 209,938,194.50	\$ 0.00	\$ 0.00	\$ 0.00	
SECTION F - OTHER BUDGET INFORMATION					
21. Direct Charges:			22. Indirect Charges:		
23. Remarks:					

INSTRUCTIONS FOR THE SF-424A

Public reporting burden for this collection of information is estimated to average 180 minutes per response, including time for reviewing instructions, searching existing data sources, gathering and maintaining the data needed, and completing and reviewing the collection of information. Send comments regarding the burden estimate or any other aspect of this collection of information, including suggestions for reducing this burden, to the Office of Management and Budget, Paperwork Reduction Project (0348-0044), Washington, DC 20503.

PLEASE DO NOT RETURN YOUR COMPLETED FORM TO THE OFFICE OF MANAGEMENT AND BUDGET. SEND IT TO THE ADDRESS PROVIDED BY THE SPONSORING AGENCY.

General Instructions

This form is designed so that application can be made for funds from one or more grant programs. In preparing the budget, adhere to any existing Federal grantor agency guidelines which prescribe how and whether budgeted amounts should be separately shown for different functions or activities within the program. For some programs, grantor agencies may require budgets to be separately shown by function or activity. For other programs, grantor agencies may require a breakdown by function or activity. Sections A, B, C, and D should include budget estimates for the whole project except when applying for assistance which requires Federal authorization in annual or other funding period increments. In the latter case, Sections A, B, C, and D should provide the budget for the first budget period (usually a year) and Section E should present the need for Federal assistance in the subsequent budget periods. All applications should contain a breakdown by the object class categories shown in Lines a-k of Section B.

Section A. Budget Summary Lines 1-4 Columns (a) and (b)

For applications pertaining to a *single* Federal grant program (Federal Domestic Assistance Catalog number) and *not requiring* a functional or activity breakdown, enter on Line 1 under Column (a) the Catalog program title and the Catalog number in Column (b).

For applications pertaining to a *single* program *requiring* budget amounts by multiple functions or activities, enter the name of each activity or function on each line in Column (a), and enter the Catalog number in Column (b). For applications pertaining to multiple programs where none of the programs require a breakdown by function or activity, enter the Catalog program title on each line in *Column* (a) and the respective Catalog number on each line in Column (b).

For applications pertaining to *multiple* programs where one or more programs *require* a breakdown by function or activity, prepare a separate sheet for each program requiring the breakdown. Additional sheets should be used when one form does not provide adequate space for all breakdown of data required. However, when more than one sheet is used, the first page should provide the summary totals by programs.

Lines 1-4, Columns (c) through (g)

For new applications, leave Column (c) and (d) blank. For each line entry in Columns (a) and (b), enter in Columns (e), (f), and (g) the appropriate amounts of funds needed to support the project for the first funding period (usually a year).

For continuing grant program applications, submit these forms before the end of each funding period as required by the grantor agency. Enter in Columns (c) and (d) the estimated amounts of funds which will remain unobligated at the end of the grant funding period only if the Federal grantor agency instructions provide for this. Otherwise, leave these columns blank. Enter in columns (e) and (f) the amounts of funds needed for the upcoming period. The amount(s) in Column (g) should be the sum of amounts in Columns (e) and (f).

For supplemental grants and changes to existing grants, do not use Columns (c) and (d). Enter in Column (e) the amount of the increase or decrease of Federal funds and enter in Column (f) the amount of the increase or decrease of non-Federal funds. In Column (g) enter the new total budgeted amount (Federal and non-Federal) which includes the total previous authorized budgeted amounts plus or minus, as appropriate, the amounts shown in Columns (e) and (f). The amount(s) in Column (g) should not equal the sum of amounts in Columns (e) and (f).

Line 5 - Show the totals for all columns used.

Section B Budget Categories

In the column headings (1) through (4), enter the titles of the same programs, functions, and activities shown on Lines 1-4, Column (a), Section A. When additional sheets are prepared for Section A, provide similar column headings on each sheet. For each program, function or activity, fill in the total requirements for funds (both Federal and non-Federal) by object class categories.

Line 6a-i - Show the totals of Lines 6a to 6h in each column.

Line 6j - Show the amount of indirect cost.

Line 6k - Enter the total of amounts on Lines 6i and 6j. For all applications for new grants and continuation grants the total amount in column (5), Line 6k, should be the same as the total amount shown in Section A, Column (g), Line 5. For supplemental grants and changes to grants, the total amount of the increase or decrease as shown in Columns (1)-(4), Line 6k should be the same as the sum of the amounts in Section A, Columns (e) and (f) on Line 5.

Line 7 - Enter the estimated amount of income, if any, expected to be generated from this project. Do not add or subtract this amount from the total project amount, Show under the program

INSTRUCTIONS FOR THE SF-424A (continued)

narrative statement the nature and source of income. The estimated amount of program income may be considered by the Federal grantor agency in determining the total amount of the grant.

Section C. Non-Federal Resources

Lines 8-11 Enter amounts of non-Federal resources that will be used on the grant. If in-kind contributions are included, provide a brief explanation on a separate sheet.

Column (a) - Enter the program titles identical to Column (a), Section A. A breakdown by function or activity is not necessary.

Column (b) - Enter the contribution to be made by the applicant.

Column (c) - Enter the amount of the State's cash and in-kind contribution if the applicant is not a State or State agency. Applicants which are a State or State agencies should leave this column blank.

Column (d) - Enter the amount of cash and in-kind contributions to be made from all other sources.

Column (e) - Enter totals of Columns (b), (c), and (d).

Line 12 - Enter the total for each of Columns (b)-(e). The amount in Column (e) should be equal to the amount on Line 5, Column (f), Section A.

Section D. Forecasted Cash Needs

Line 13 - Enter the amount of cash needed by quarter from the grantor agency during the first year.

Line 14 - Enter the amount of cash from all other sources needed by quarter during the first year.

Line 15 - Enter the totals of amounts on Lines 13 and 14.

Section E. Budget Estimates of Federal Funds Needed for Balance of the Project

Lines 16-19 - Enter in Column (a) the same grant program titles shown in Column (a), Section A. A breakdown by function or activity is not necessary. For new applications and continuation grant applications, enter in the proper columns amounts of Federal funds which will be needed to complete the program or project over the succeeding funding periods (usually in years). This section need not be completed for revisions (amendments, changes, or supplements) to funds for the current year of existing grants.

If more than four lines are needed to list the program titles, submit additional schedules as necessary.

Line 20 - Enter the total for each of the Columns (b)-(e). When additional schedules are prepared for this Section, annotate accordingly and show the overall totals on this line.

Section F. Other Budget Information

Line 21 - Use this space to explain amounts for individual direct object class cost categories that may appear to be out of the ordinary or to explain the details as required by the Federal grantor agency.

Line 22 - Enter the type of indirect rate (provisional, predetermined, final or fixed) that will be in effect during the funding period, the estimated amount of the base to which the rate is applied, and the total indirect expense.

Line 23 - Provide any other explanations or comments deemed necessary.